Menu

Syndromes and Diseases, by Body Systems

- Bone, Muscle & Joint Infections
  - Septic Bursitis
  - Diskitis And Vertebral Osteomyelitis
  - Osteomyelitis
  - Infections In Bone Or Joint Prostheses
  - **SEPTIC ARTHRITIS**
  - Native Joint
  - Prosthetic Joint
- Cardiovascular
- Central Nervous System
- GI & Intraabdominal
- Head And Neck
- Lungs & Mediastinum
- Skin & Soft Tissue Infections
- Systemic Infections
- Urogenital

Organisms

- Bacteria
- Fungi
- Other Pathogens
- Parasites
- Viruses

Welcome to CDSS

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OSTEOMYELITIS

Obtain material for gram-stain and culture before initiating antimicrobials.
Blood and bone cultures are ideal.

Antibiotics should be held if patient does not have adjacent skin/soft tissue infection
Duration of treatment and route will vary with differences in location of
infection, infecting organism, and host comorbidities.

Test Edit

Empirical Treatment Regimens

Empirical Therapy For Osteomyelitis

> For Patients With Infections Associated With Prosthetic Materials See
Infections In Bone Or Joint Prostheses

References:
Mandell chapter: Osteomyelitis
Sanford Guide: osteomyelitis, foot bone
Page ID: osteomyelitis

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OSTEOMYELITIS > EMPIRICAL THERAPY FOR OSTEOMYELITIS

EMPIRICAL THERAPY FOR OSTEOMYELITIS

Recommend Infectious Disease consultation.
Bone culture is essential and diagnostic. If patient is not clinically ill and adiagnostic procedure is planned, antibiotics can be held until after the
procedure.

Therapy should be modified when culture results and patient response to therapy are known.

Duration of treatment is 4 to 6 weeks.
Empiric Therapy

Therapy

Preferred:
Ceftriaxone 2 gm IV q24h M

History of Pseudomonas in wound:
Piperacillin-tazobactam 4.5 gm IV q6h R

If MRSA history:
<ADD> Vancomycin 15 mg/kg IV q12h

Oral switch recommendations based on culture results

Treatment for MSSA and streptococci:
Cefadroxil 1000 mg PO q12h R

Treatment for MRSA:
Doxycycline 100 mg PO q12h DI
< OR >
Trimethoprim-sulfamethoxazole 1 DS tablet PO q12h R,DI

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SEPTIC ARTHRITIS (NATIVE JOINT)

> For arthritis associated with a PROSTHETIC JOINT see
Infections in bone or joint prostheses

> For arthritis associated with a BITE WOUND see
Bite wounds

> For arthritis associated with TICK EXPOSURE in an endemic area, consider
Lyme disease

Septic arthritis native joint:

Aspirate the joint for gram stain and culture. Use culture results to guide
definitive therapy Rule out N. gonorrhea in sexually active patients

Duration of therapy:
Duration of therapy is based on pathogen and clinical response. Please consult

ID for further treatment recommendations

References:
Mandell chapter: Infectious arthritis of native joints
Sanford keyword: Septic arthritis
Page ID: septic-arthritis-nat-joint

Therapy

Empiric antibiotic recommendations:
Vancomycin 15mg/kg IV q 12 hours [R]

History of Pseudomonas and/or severe immunosuppression:
Cefepime 2 gm IV q8h ([R]
<AND> Vancomycin 15 mg/kg IV q12h [R]

If synovial fluid gram stain with Gram negative rods:
Ceftriaxone 2 gm IV q 24 h